The Relationship Between Depression and Anxiety and Symptoms of Misophonia in the General Population

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INTRODUCTION

- Misophonia is a neuropsychiatric condition where aversive emotional responses and increased physiological arousal are produced following certain trigger sounds.
- There is mounting research that suggests those who experience clinical misophonia symptoms face mental health challenges.
- The relationship between misophonia and mental health has not been adequately assessed in the general population.

RESEARCH QUESTION

- Are symptoms of misophonia associated with depression and anxiety in the general population?

METHOD

- 1,403 undergraduates were recruited through Wilfrid Laurier University's Psychology Research Experience Program (PREP).
- The Duke-Vanderbilt Misophonia Questionnaire (DVMQ) was used to measure and categorize misophonia symptoms at non-clinical, subclinical, and clinical levels.
- The Duke Misophonia Questionnaire (DMQ) was used to assess severity and caseness of misophonia, as well as coping abilities.
- The Beck Depression Inventory (BDI) and Beck Anxiety Inventory (BAI) were used to determine symptoms of depression and anxiety.

TABLE 1.	Gender	Age	DMQ Coping	DMQ Severity	BDI Total	BAI Total
Mean (SD)	1073F, 304M, 21O	19.5 (2.6)	21.6 (17.9)	22.6 (19.8)	14.4 (10.0)	18.1 (13.4)
Range	--	17-59	0-82	0-92	0-57	0-63

TABLE 1. FOR GENDER, F = FEMALE, M = MALE, O = OTHER.

RESULTS

FIG.1

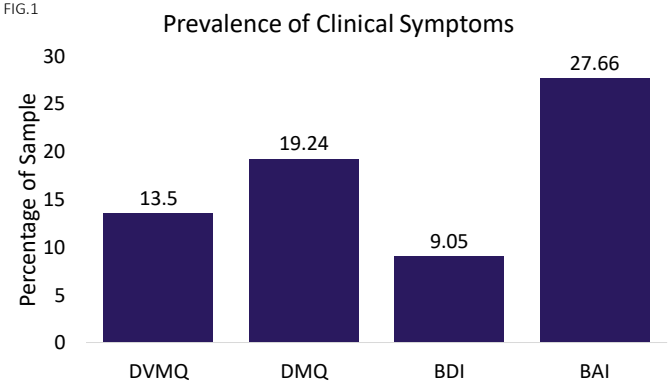

FIGURE 1. Percentage of Clinically Significant Scores. Cut-off scores: DVMQ = N/A; DMQ = 41.4; BDI = 29; BAI = 26.

FIG.2

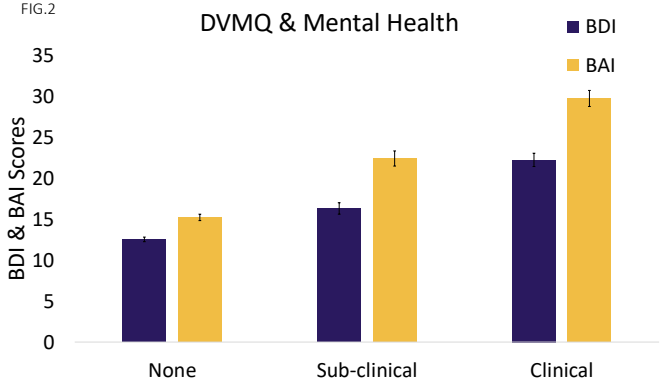

FIGURE 2. DVMQ & Mental Health. A main effect of DVMQ score was found on BDI $F(2,316) = 70.9$, $p < .001$ and BAI $F(2,324) = 111$, $p < .001$.

FIG.3

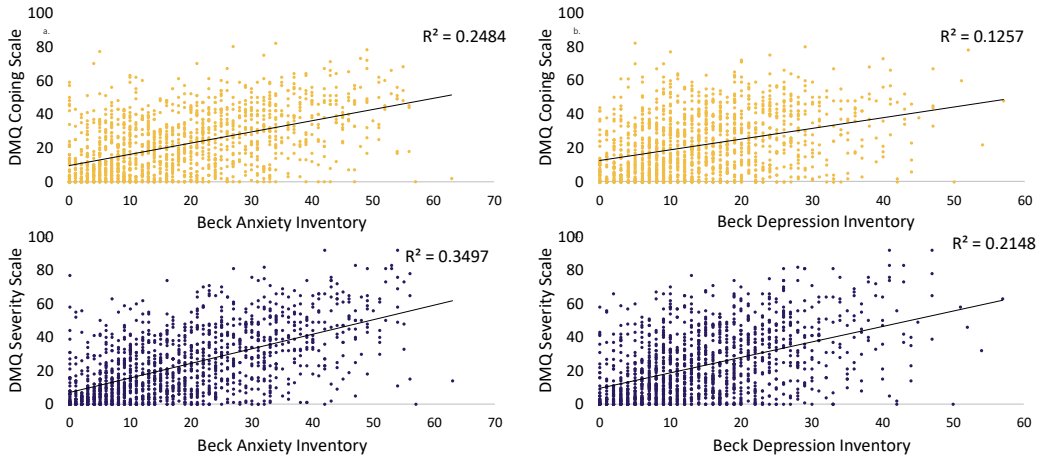

FIGURE 3. BDI & BAI Correlations. BDI and BAI in relation to DMQ Coping and Severity scores. *A:* $r(1401) = .50$, $p < .001$, *moderate; B:* $r(1401) = .36$, $p < .001$, *moderate; C:* $r(1401) = .59$, $p = <.001$, *moderate; D:* $r(1,401) = .46$, $p < .001$, *moderate.*

SCAN ME

DISCUSSION

- Robust correlations suggest misophonia symptoms are related to increased depression and anxiety. This relationship exists in the general population, independent of a clinical misophonia diagnosis.
- University students may be at higher risk for mental health challenges in relation to misophonia as they have less control of the noises produced in their regular environments (e.g., dining halls, lectures, large-scale examinations).
- As our data is correlational, it is unclear if misophonia symptoms are a causal factor for increased depression & anxiety.
- Research should focus on strategies for treating and managing misophonia as it may negatively impact daily functioning.

This research was supported by the Wilfrid Laurier University Research Support Fund.